

# SENATE . . . . . No. 858

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## The Commonwealth of Massachusetts

PRESENTED BY:

*Lydia Edwards*

*To the Honorable Senate and House of Representatives of the Commonwealth of Massachusetts in General Court assembled:*

The undersigned legislators and/or citizens respectfully petition for the adoption of the accompanying resolve:

Resolve relative to pharmacy deserts.

PETITION OF:

NAME:

*Lydia Edwards*

DISTRICT/ADDRESS:

*Third Suffolk*

# SENATE . . . . . No. 858

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By Ms. Edwards, a petition (accompanied by resolve, Senate, No. 858) of Lydia Edwards that provisions to assess the supply, distribution and capacity of pharmacy and pharmacological services. Health Care Financing.

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## The Commonwealth of Massachusetts

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In the One Hundred and Ninety-Fourth General Court  
(2025-2026)  
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Resolve relative to pharmacy deserts.

1            ~~Resolved~~, SECTION 1. (a) The office of health resource planning established in section  
2    22 of chapter 6D of the General Laws, inserted by section 22 of chapter 343 of the Acts of 2024,  
3    shall conduct a focused assessment on supply, distribution and capacity of pharmacy and  
4    pharmacological services pursuant to subsection (b) of said section 22. The office, when  
5    conducting its focused assessment, shall also identify the number of existing and potential  
6    pharmacy deserts in the commonwealth and conduct an analysis of their impact or potential  
7    impact on access to pharmacy and pharmacological services for residents located in the  
8    identified areas. For the purposes of this section, a “pharmacy desert” shall mean an area where  
9    there is no or limited access to pharmacies due to factors such as: (i) geographic location,  
10   specifically areas where the nearest pharmacy is more than 1 mile away in urban areas, more  
11   than 5 miles away in suburban areas and more than 10 miles away in rural areas; (ii) distance and  
12   travel time, defined as travel time exceeding 15 minutes by car or 30 minutes by public  
13   transportation; and (iii) limited access to transportation, both public and private, including areas

with infrequent public transit services or where at least 20 per cent of the population lacks access to private vehicles.

(b) Not later than September 1, 2026, the office shall present to the board of the health policy commission its findings based on the focused assessment conducted under subsection (a) and file a report of its findings with the clerks of the senate and house of representatives, the house and senate committees on ways and means, the joint committee on health care financing, the center for health information and analysis, the health policy commission and the department of public health. In addition to the findings required by paragraph (2) of subsection (b) of section 22, report of the office shall analyze the impact or potential impact of pharmacy deserts identified by the focused assessment, including, but not limited:

(i) an assessment on impacted neighborhoods and patient populations;

(ii) an assessment on the impact of pharmacy deserts on access to medications and health care outcomes;

(iii) an assessment of the geographical and financial barriers to obtaining medications faced by individuals living in pharmacy deserts;

(iv) an assessment of the average distance and travel time to a pharmacy from an impacted neighborhood, and the transportation options available;

(v) an assessment on the impact of pharmacy deserts on overall health care costs, including the costs of emergency department visits and hospitalizations;

(vi) an assessment on the impact of pharmacy benefit manager business practices in contributing to the closures of pharmacies across the commonwealth; and

35 (vii) policy recommendations to address current pharmacy deserts and limit the creation  
36 of new ones.